

The Current State of Evusheld (tixagevimab/cilgavimab) in the UK

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Evusheld is a combination of two long-acting antibodies (tixagevimab and cilgavimab). It's a drug designed to protect clinically vulnerable people against Covid in cases where vaccines don't work. A good example is people who have been on the chemotherapy drug Rituximab, which causes much worse Covid outcomes and also reduces vaccine efficacy. Another example is people with primary or secondary immunodeficiency. Evusheld is given prophylactically (that is: in advance) to stop people in this situation developing severe Covid in the first place.

This drug represents the biggest lifeline yet for vulnerable people who remain shielding.

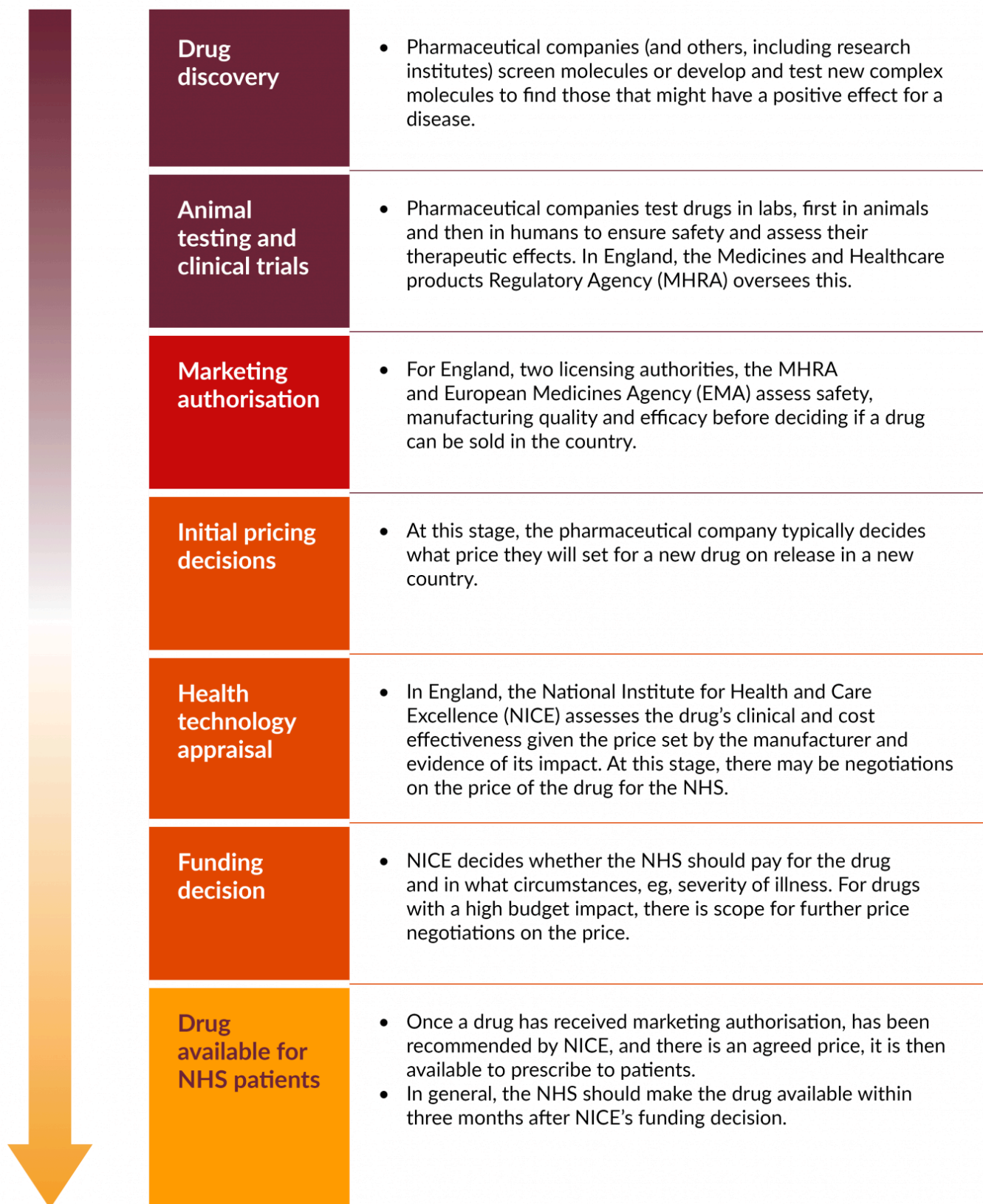
Evusheld was authorised by the UK's MHRA on the 17th March 2022. This means that it met the UK regulatory standards of safety, quality and effectiveness. As a result, we were told, "the decision to grant approval for this treatment was endorsed by the government's independent expert scientific advisory body, the Commission on Human Medicines, after carefully reviewing the evidence". You would think this would mean that vulnerable people would, by now, have access to this lifesaving drug.

However, as of the end of April 2022, it is still *impossible* to get Evusheld in the UK, either on the NHS, or privately. The Daily Mail ran an article highlighting this. However, they also implied that it was available privately if you could pay. This is *not true*. Private hospitals and clinics are unable, at

the moment, to procure Evusheld.

What's going on? The process for approving medications for NHS usage in the UK is long and complex. The King's Fund [have a good primer](#). Evusheld is currently at the "Marketing Authorisation" stage. There are several more hurdles before it gets approval.

Overview of the development, pricing, licensing and appraisal processes for new drugs



Several MPs have raised this issue in parliament, but we need more help. [Vicky Foxcroft asked about the drug](#) at Prime Minister's Questions. [Caroline Lucas has also raised the issue](#). My MP, Craig Mackinlay, has written to our local Clinical Commissioning Group. Please do [ask your MPs](#) to do what they can.

Update 29th April

My local Clinical Commissioning Group replies:

“Although Evusheld 150 mg / 150 mg solution for injection, tixagevimab/cilgavimab has been approved by the MHRA, there is currently no national policy decision yet (this would need to come from NHS England (NHSE) for approval in the NHS). [...] Unfortunately, I am unable to say when we will receive information as to the policy decision from NHSE.”

Update on 4th May

I wrote the following letter to [Lord Mendelsohn](#), who has consistently spoken out on our behalf:

Dear Lord Mendelsohn,

I am a Professor of English at the University of London, but also a severely immunocompromised patient given the chemotherapy drug, Rituximab, to treat a series of serious autoimmune conditions.

I wanted to write, first, to thank you for your advocacy in recent days for the immunocompromised of this country. I remain shielding, unable to go anywhere or do anything because my life remains at serious threat. We have been under near-constant house arrest for over two years now and life is very difficult. I am so grateful for those in our political system, like you, who have spoken on our behalf. It gives me hope that the system may still help us.

I also wanted to write, though, to ask that you continue advocating strongly for the UK government to purchase Evusheld (tixagevimab/cilgavimab), the new preventative COVID-19 medication from AstraZeneca. This drug has been purchased by every developed nation in the world to protect those who can't mount a response to the Covid vaccines... except the UK. It is **not** even available privately as AstraZeneca are only selling it to national healthcare systems.

Hence, a way back to a proper life is available -- but out of reach for anyone in the UK, at present. Whatever you can do to help would be more appreciated than I can say.

We are told we have to "live with Covid". However, I, and many others in my situation, cannot. We would have to die with it.

Yours sincerely,

Martin Eve

Update on 5th May

I received a response from another private clinic, whose pharmacist contacted AstraZeneca directly: "I received a very limited response from AstraZeneca, this product is only available to NHS patients, I am afraid".

Of course, it is *not* available to NHS patients. So it's not available to anyone.

Update on 6th May

One MP replied: "ministers have said that they are unable to provide further information as it is commercially sensitive. I am told that any announcement on procurement would be made after an agreement is reached with companies". This looks, then, as though the reason it's not available is

that ministers are haggling over the price of a really not-very-expensive treatment.

One of the Lords to whom I wrote responded, generously: “We have an option which works and is available in other 25 countries and works exceptionally well. [...] The CEV [Clinically Extremely Vulnerable] community has been let down by the whole health structure including the scientists and researchers. [...] I am really at a loss to know whether it is heartlessness or incompetence that stops the UK making Evusheld available, but this should be an affordable and easy process to deliver and the fact that we aren’t doing it just beggars’ belief.”

Update on the 30th May

An independent advisory medical group today [released a report](#), “Defining the highest-risk clinical subgroups upon community infection with SARS-CoV-2 when considering the use of neutralising monoclonal antibodies (nMABs) and antiviral drugs: independent advisory group report”. The report defines those who remain still at high risk of death or serious illness from Covid.

Personally, I meet three of the criteria (people who are on biologics or small molecule JAK-inhibitors / people who are on corticosteroids (equivalent to greater than 10mg per day of prednisolone) / any person with secondary immunodeficiency receiving, or eligible for, immunoglobulin replacement therapy)) but this will cover many people.

In all, this report feels as though Evusheld and other prophylactics might be getting closer in the UK, but there is still no availability.

Update on the 4th June

Canned responses from politicians are now coming thick and fast. Many MPs are claiming “commercial sensitivity” as a reason not to discuss the Evusheld situation, even though the CEO of AstraZeneca has said that there is no such non-disclosure agreement because the government is not negotiating to buy Evusheld. Meanwhile, the Parliamentary Under-Secretary of State for Innovation at the Department of Health, Lord Kamall, replied to a disabled person re. Evusheld that “the evidence points to treatment rather than prophylaxis”. This is clearly nonsense. The government’s central strategy for Covid has been *vaccination*, which is prophylaxis. The argument here is basically that there will be prophylaxis for most of the population, but the vulnerable and

disabled will not be afforded the same protection. This group, it seems, must run the risk of contracting a possibly fatal illness before they will be treated. It is hard to see this as anything other than discrimination against a vulnerable group.

This post was last updated on the 4th June 2022. It was originally written on the 29th April. I will update this post if anything changes.